Cascade Health

AI prior-authorization workflow for specialty clinics

Series A, 2026

Problem

Specialty clinics wait 12 days on average for prior authorization.

31% of first submissions are denied for missing documentation.

Each denial costs a clinic $118 in staff rework.

Solution

Cascade drafts payer-specific authorization packets from the EHR chart.

Missing clinical evidence is flagged before submission, not after denial.

Approvals land in 3.2 days on average across live sites.

Market

US prior-authorization spend: $3.1B in administrative labor annually.

Bottoms-up: 38,000 specialty clinics x $60K ACV = $2.3B serviceable market.

CMS interoperability rules mandate electronic prior auth by 2027.

Traction

ARR $1.8M, up 3.4x year over year.

41 clinic customers; 96% gross revenue retention.

First-pass approval rate 89% versus 69% industry baseline.

Team

CEO Dr. Priya Raman: 9 years practicing oncologist, ex-UnitedHealth clinical ops.

CTO Alex Fournier: ex-Epic integration lead, 12 years health IT.

Head of ML from Tempus; 22 FTEs.

Business Model

Per-provider SaaS subscription, $60K average ACV.

Gross margin 68% at current scale.

Expansion: denial appeals and payer analytics modules.

Financials

Monthly burn $210K; runway to Q4 2027 post-raise.

CAC payback 11 months; NRR 124%.

Services 14% of revenue and falling.

The Ask

Raising $12M Series A led round.

Use of funds: payer integrations, 3 new specialties, compliance team.

Contact

Dr. Priya Raman, CEO

priya@cascadehealth.example

Denver, CO